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NANOTECH Bio Lab
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Center Name : **NANOTECH BIO LAB - HEAD OFFICE**	Sample Collected On : 24.03.2026 13:16	Patient UID No : NT2603076567
Patient Name : **Mrs THENMOZHI S**	Sample Accepted On : 24.03.2026 13:16	Lab Number : 129723
Age and Gender : 43 Y /F	Results Approved : 24.03.2026 17:36	Sample UID No :
Referring Doctor : **DR. SENTHIL O. P. M.B.B.S MS.,(General Surgery)**	Printed On : 25.03.2026 18:18	**2603240079**

Test Name	Value	Units	Comments	Previous Values	Reference Range
BIOCHEMISTRY					
C Reactive Protein (CRP)	**6.4**	mg/L	*		UPTO 6.00 mg/L [Turbidimetry Method]

Note:The CRP response frequently precedes clinical symptoms such as infections, inflammatory diseases and malignant neoplasms. Alterations are detectable within 6 to 8 hours and the peak value is reached within 24 to 48 hours. Levels of up to thousandfold the normal value are associated with severe stimuli such as myocardial infarction, major trauma, surgery, or malignant neoplasms. CRP levels can be elevated in the later stages of pregnancy, birth control pills or hormone replacement therapy (i.e., estrogen) and obesity. Significantly decreased CRP values may be obtained from samples taken from patients who have been treated with carboxypenicillins. In very rare cases,gammopathy, in particular type IgM (Waldenström's macroglobulinemia) and patients who have been treated or recieved monoclonal mouse antibodies, may cause unreliable results. CRP determination may replace the classical determination of Erythrocytes Sedimentation Rate (ESR), due to its prompt response to changes in disease activity (CRP increases sooner and then decreases more rapidly than the ESR) and its good correlation to ESR.

It aids in evaluation of stress, Trauma, Infection, Inflammation, Surgery and associated diseases.

IMMUNO ASSAY					
ENA PROFILE QUANTITATIVE- IgG-25					
Nucleosome - Ab	**0**	U/ml	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic marker for systemic lupus Erythematosus(SLE) sensitivity of 56-90%

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ENA PROFILE QUANTITATIVE- IgG-25					
ds DNA - Ab	**0**	U/ml	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic marker for systemic lupus Erythematosus(SLE) frequently detected (>95%) in SLE with renal involvement, detected in >50-70% in active SLE without renal involvement and in <40% in inactive SLE. Associated with the severity of SLE.
Histones - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Can be detected in a number of autoimmune diseases, especially rheumatic disorders:Systemic lupus erythematosus(SLE) (50-80%). Drug-induced Lupus (DIL) (92-95

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ENA PROFILE QUANTITATIVE- IgG-25					
Sm - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic marker for Systemic Lupus Erythematosus(SLE) diagnostic specificity of 99% for SLE. Diagnostic sensitivity of 5-40% for SLE.
RNP 68kD/A/C Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic criterion of mixed connective tissue diseases (MCTD). Highly specific and extremely sensitive(100%) in the absence of Sm and dsDNA antibodies.

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ENA PROFILE QUANTITATIVE- IgG-25					
Sm/RNP-Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic marker for Systemic Lupus Erythematosus(SLE). Diagnostic specificity of 99% for SLE. Diagnostic sensitivity of 5-40% for SLE. RNP 68kD/A/C:Diagnostic criterion of mixed connective tissue diseases(MCTD).
SSA/Ro 60kD - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic marker and classification criterion for Sjogren syndrome(SjS). Found in 96% of patients with primary SjS. Found in 80% of patients with secondary SjS.

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ENA PROFILE QUANTITATIVE- IgG-25					
SSA/Ro 52kD - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Found in various autoimmune diseases such as SLE(23%, SjS(17-63%), Systemic Sclerosis(SSC)(20%), Rheumatoid arthritis(RA)(8%), Primary billary cirrhosis(PBC)(28%), Autoimmune hepatitis (17%). Often detectable in myositis patients with aminoacyl-tRNA synthetase SRP antibodies , PM/Scl antibodies and Jo-1 antibodies
SSB - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic marker for Sjogren Syndrome(SjS).

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Test Name	Value	Units	Comments	Previous Values	Reference Range
ENA PROFILE QUANTITATIVE- IgG-25					
Scl-70 - Ab	**1**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic marker for systemic sclerosis(SSc). Diagnostic specificity of 99%, Sensibility of 10% for limited SSc and upto 65% for diffuse SSc.

Antibodies to Scl-70 (DNA topoisomerase I) are detected in nearly 75% patients with Progressive Systemic Sclerosis (PSS). These antibodies can also be detected in 20-59% patients of connective tissue disorders and 13% patients with CREST syndrome. Patients of Scleroderma with Scl-70 antibody positivity are associated with diffuse cutaneous involvement, increased frequency of pulmonary fibrosis and high mortality.

Ku - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Found in 23% of patients with primary pulmonary hypertension, 23% of patients with SLE, 14% of patients with systemic sclerosis and 33% of patients with an overlap syndrome with myositis.

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ENA PROFILE QUANTITATIVE- IgG-25					
PM-Scl100 - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic marker for connective disease with myositis and symptoms of systemic sclerosis. Diagnostic specificity of 50-70% for polymyositis/scleroderma overlap syndrome
Mi-2 - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic Marker for idiopathic myositis, with a diagnostic sensitivity of 4 -18%. Detectable in 15-31% of patients with adult dermatomyositis and in 10-15% of those with juvenile dermatomyositis Prognostic marker for a relatively mild clinical course, but associated with an increased risk of cancer Detectable in early stage of myositis development

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ENA PROFILE QUANTITATIVE- IgG-25					
Jo-1 - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic Marker for idiopathic autoimmune myositis, Diagnostic specificity of 100%, Diagnostic sensitivity of 24-30% for autoimmune idiopathic myositis.
PL-7 - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic Marker for idiopathic myositis sensitivity of 2-3%. Highly associated with the presence or development of interstitial lung disease(ILD).
PL-12 - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic Marker for idiopathic myositis, sensitivity of 2-3%. Highly associated with the presence or development of interstitial lung disease(ILD).

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ENA PROFILE QUANTITATIVE- IgG-25					
SRP - 54 Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic Marker for polymyositis, specificity of 100%, sensitivity of 4-6%. Differential diagnostic and prognostic marker:rapid-progressive proximal muscle weakness.
Ribosomes Po - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic Marker for SLE(>99%), found in 10-35% of SLE patients. Associated with diseases activity (Prognostic marker) may be detectable prior to SLE manifestations(predictive marker).

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ENA PROFILE QUANTITATIVE- IgG-25					
CENP-A/B - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Diagnostic Marker for Systemic sclerosis (SSc). Sensitivity of 57-82% for patients with CREST Syndrome(or other limited cutaneous forms of SSc) and in 3-12% of patients with diffuse cutaneous forms of SSc. Detectable in 10-30% of patients with primary billiary cirrhosis(PBC).
PCNA - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Highly specific for SLE but rarely found (3-7%).

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ENA PROFILE QUANTITATIVE- IgG-25					
Sp100 - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Anti-Sp100 antibodies are specific (97% for primary billiary cholangitis(PBC) with a diagnostic sensitivity of 20-40%. These auto antibodies are found relatively often (48%) in the group of AMA negative patients with a clinically and histologically proven PBC.
GP210 - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Anti-gp210 antibodies are highly specific for primary billiary cholangitis(PBC) and are detectable by enzyme immunoassay in 10-45% of PBC patients with a specificity of 99.5%.

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ENA PROFILE QUANTITATIVE- IgG-25					
M2 recombinant - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. AMA-M2 recombinant are directed against proteins of theE2 components of the 2-oxoacid dehydrogenase family of enzyme complexes (2-OACD). Anti-M2/nPDC aremarker antibodies of primary billiary cholangitis (PBC) and are detectable in nearly 95% of cases.
M2 Native - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. Anti-M2/nPDC are marker antibodies of primary billiary cholangitis (PBC) and are detectable in nearly 95% of cases.
f-actin - Ab	**0**	U/mL	*		< 6:Negative, 6-12:Equivocal, >12:Positive, METHOD:IMMUNO DOT ASSAY. High titers of anti-F-actin are marker antibodies for autoimmune hapatitis(AIH).

HAEMATOLOGY

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Sangamam Towers,
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52, Dindigul Road, Palani.
95858 98555
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NANOTECH Bio Lab
SINCE 2018
Complete Health Solution

Center Name : **NANOTECH BIO LAB - HEAD OFFICE**	Sample Collected On : 24.03.2026 13:16	Patient UID No : NT2603076567
Patient Name : **Mrs THENMOZHI S**	Sample Accepted On : 24.03.2026 13:16	Lab Number : 129723
Age and Gender : 43 Y /F	Results Approved : 24.03.2026 17:36	Sample UID No :
Referring Doctor : **DR. SENTHIL O. P. M.B.B.S MS.,(General Surgery)**	Printed On : 25.03.2026 18:18	**2603240079**

Test Name	Value	Units	Comments	Previous Values	Reference Range
COMPLETE BLOOD COUNT (CBC)					
TOTAL WBC COUNT	**7210**	Cells/Cumm	*		Adult - 4000 -10000
Ref. Ranges for Childrens : (Birth 10000-26000, 0-3 days 7000-23000, 4-7 days 6000-22000, 8-14 days 6000-22000, 15day-1 mon 5000-19000, 2mon-3mon 5000-15000, 4-6 mons 6000-18000, 7mons-1 yr 6000-16000, 2-6 yrs 5000-15000, 7-12 yrs 5000-13000, Reference:Dacie and Lewis,)					
TOTAL RBC COUNT	**4.08**	Million/uL	*		4.50 - 5.50
Ref. Ranges for Childrens : (Birth 5.0-7.0, 0-3 days 4.0-6.6, 4-7 days 3.9-6.3, 8-14 days 3.6-6.2, 15day-1 mon 3.0-5.4, 2mon-3mon 3.1-4.3, 4-6 mons 4.1-5.3, 7mons-1 yr 3.9-5.1, 2-6 yrs 4.0-5.2, 7-12 yrs 4.0-5.2)					
HAEMOGLOBIN [HB %]	**10.5**	gm/dl	*		12.0-15.0 (Adult)
Ref. Ranges for Childrens : (Birth 14.0-22.0, 0-3 days 15-21, 4-7 days 17.1-17.9, 8-14 days 16.1-16.9, 15day-1 mon 11.5-16.5, 2mon-3mon 9.4-13.0, 4-6 mons 11.1-14.1, 7mons-1 yr 11.1-14.1, 2-6 yrs 11.0-14.0, 7-12 yrs 11.5-15.5					
HEMATOCRIT (PCV)	**32.2**	%	*		36 - 46
TOTAL PLATELET COUNT	**310000**	Cells/uL	*		Adult : 150000 - 410000
Ref. Ranges for Childrens : (Birth 100000-450000, 0-3 days 210000-500000, 4-7 days 160000-500000, 8-14 days 170000-500000, 15day-1 mon 200000-500000, 2mon-3mon 210000-650000, 4-6 mons 200000-550000, 7mons-1 yr 200000-550000, 2-6 yrs 200000-490000, 7-12 yrs 170000-450000					
MCV	**78.8**	fl	*		76 – 96
MCH	**25.7**	pg	*		27.0 - 32
MCHC	**32.7**	g/dl	*		31.5-34.5
MPV	**8.7**	fL	*		6.5 - 12.0
DIFFERENTIAL WBC COUNT					
NEUTROPHIL %	**53**	%	*		40 – 80
LYMPHOCYTE %	**38**	%	*		20-40

A.Gomathy(DMLT)

Dr.S.Varun Prakash M.D., (Micro)
(Consultant Microbiologist)

P.Thangaraj M.Sc(Bio-Chemistry)
(Chief of Lab Service)

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Test Name	Value	Units	Comments	Previous Values	Reference Range
COMPLETE BLOOD COUNT (CBC)					
MONOCYTES	**05**	%	*		2.0-10.0
EOSINOPHIL	**04**	%	*		1.0-6.0
BASOPHILS %	**00**	%	*		0.0 - 1.0

Comments: Please Correlate with Clinical Conditions.

~~~ END OF REPORT ~~~

A.Gomathy(DMLT)

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(Consultant Microbiologist)

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